NCCP 00692 · clinician draft

# Weekly cetuximab + FOLFOX-4

**Supportive prescription draft for review.** This is a regimen template. It is not an issued patient prescription; individual review and completion are required.

## Day 8 · weekly cetuximab only

**Active medicines:** Cetuximab

**Emetogenic risk:** Low

| Medicine | Dose / route | When |
|---|---|---|
| Chlorphenamine | 10 mg IV bolus | At least one hour before cetuximab<br>Cetuximab premedication remains applicable. |
| Dexamethasone | 8 mg IV bolus | At least one hour before cetuximab<br>Reconcile low-risk antiemesis with this corticosteroid; do not duplicate steroids. |
| Metoclopramide | 10 mg PO up to three times daily as required | Rescue only as prescribed<br>No automatic delayed steroid course is carried over from Day 1 chemotherapy. |

### Additional phase-specific support

- Cetuximab only: no irinotecan, oxaliplatin, fluorouracil or atropine is scheduled on Day 8.
- Continue skin care, electrolyte monitoring, infusion precautions and the urgent contact plan.
- Low-risk antibody-only antiemesis is distinct from Day 1 moderate-risk chemotherapy prophylaxis.
- NCCP 00692 v2 p1 permits omitting observation after two tolerated cetuximab doses, whereas current EMA Erbitux SmPC sections 4.2/4.4 require monitoring during and for at least one hour after every infusion. Reconcile this conflict with oncology pharmacy/local policy; do not automatically waive monitoring. Chemotherapy must start no earlier than one hour after cetuximab ends.

**Before signing:** verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

NCCP 00692 v2 · p6 · NCCP NCIS antiemetics V8 low-risk row

Sources checked 2026-11-05 · SACTCheck v0.84.36 · Regimen-level drafts; prescriber and oncology-pharmacy verification required